

Sources Sought Notice (SSN)
Federal Services System Support and Oversight (FSSSO)

**THIS IS NOT A FORMAL REQUEST FOR QUOTE (RFQ) AND DOES NOT
COMMIT THE CENTERS FOR MEDICARE & MEDICAID SERVICES (CMS)
TO AWARD A CONTRACT NOW OR IN THE FUTURE**

Purpose of Notice

This is a SOURCES SOUGHT NOTICE (SSN) issued for the purpose of planning and performing market research. The purpose of this Notice is to determine the availability of small businesses (e.g., 8(a), Service-Disabled Veteran Owned Small Business, HUBZone Small Business, Small Disadvantaged Business, Veteran-Owned Small Business, and Women-Owned Small Business) on the General Services Administration (GSA) Multiple Award Schedule (MAS), **Special Item Number (SIN) 541611 Management and Financial Consulting, Acquisition and Grants Management Support, and Business Program and Project Management Services or SIN 54151S Information Technology Professional Services** that have the capability to support CMS on all activities associated with providing technical support services that will enhance Assistants' knowledge and skills to facilitate enrollment in insurance coverage through the Affordable Care Act (ACA) and the capability to support the technical implementation of the No Surprises Act (NSA) and related operational provisions that address consumer price transparency.

The information gathered from this market research will help the Government determine an appropriate acquisition strategy, including whether a small business set-aside is possible. Interested contractors must submit a capability statement to demonstrate their ability to perform the requirements stated below. **No reimbursement will be made for any costs associated with providing information in response to this announcement and any follow-up information requests.** Respondents will not be notified of the results of the capability assessment. All information submitted in response to this announcement must arrive on or before the closing date. CMS reserves the right to contact respondents as necessary.

Background

On March 23, 2010, the President signed into law the Patient Protection and Affordable Care Act (P.L. 111-148). On March 30, 2010, the Health Care and Education Reconciliation Act of 2010 (P.L. 111-152) was signed into law. The two laws are collectively referred to as the Affordable Care Act. The ACA creates competitive private health insurance markets, or Federally Facilitated Marketplaces (FFMs) that have given millions of Americans and small businesses access to affordable health coverage. Marketplaces help individuals and small employers shop for, select, and enroll in high quality, private health plans that fit their needs at competitive prices. By providing a place for one-stop shopping, Marketplaces make purchasing health insurance easier and put greater control and more choice in the hands of individuals in the Marketplaces.

Consumers applying for coverage through the Marketplace can receive application assistance from in-person Assisters. In-person Assisters help consumers obtain eligibility determinations and help them through plan selection, thus increasing the number of consumers with health insurance coverage. One of the primary functions of in-person Assisters is to facilitate enrollment into healthcare coverage by obtaining a determination for advanced payment of premium tax credit, cost-sharing reductions, Medicaid, Children's Health Insurance Program (CHIP), and the Basic Health Program from the Marketplace. To facilitate these processes, in-person Assisters require training that provides a basic set of knowledge of health insurance and various forms of coverage as well as an introduction into the mechanics of obtaining Marketplace Eligibility and Enrollment determinations. Additionally, Assisters need an array of resources to rely upon to ensure that they are adequately prepared to support consumers as quickly as possible.

Enrollment assistance helps ensure that as many consumers as possible have the understanding needed to apply and enroll in health coverage. For the purposes of this SSN, "in-person Assisters" may include Navigators, Certified Application Counselors (CACs), and Agents and Brokers. This work would focus on in-person Assisters in FFM and the CMS IT systems that support them and the new Patient-Provider Dispute Resolution (PPDR) and Independent Dispute Resolution (IDR) processes. Much of the in-person Assister support content is developed by this task order then automated, stored and made accessible on the Marketplace Assister Technical Support (MATS) Salesforce instance. The MATS Salesforce instance provides automated CAC application processing, inquiry response and tracking, assister Web Based Training (WBT) ID generation and storage, and supplemental interactive microlearning trainings. This work will also support in-person assister help desk support, the development and provision of Assister continuing education programs, and the maintenance of assister email inbox operations. The MATS Salesforce instance is also currently under development to include a module for an automated assister community tool called the Marketplace Assister Community (MAC) module. This module supports in-person assister consumer efforts with the Public Health Emergency (PHE) unwinding of Medicaid eligibility expansion by enabling Assisters to leverage enrollment data and automate enrollment assistance reporting to CMS.

Another essential support service provided to FFM Assisters by this task order is the annual delivery of an estimated twenty hours of interactive, mobile-compatible, Web Based Training (WBT) curricula that is uploaded to the Center for Consumer Information and Insurance Oversight (CCIIO) IT system called the Marketplace Learning Management System (MLMS). The MLMS system is hosted and made securely and publicly available on CMS' enterprise-wide Identity Management (IDM) portal. MLMS verifies the in-person assister IDs that are generated and stored on the MATS Salesforce system and connected with MLMS through an Application Programming Interface (API). The MATS Salesforce instance is one of many CCIIO Salesforce instances that must maintain its compliance with the CCIIO-wide Customer Relationship Management System (CCRMS) Salesforce system security protocol boundary. To maximize efficiencies for CCIIO, this task order also enhances and maintains the Marketplace Plan

Management Group's (MPMG) System for Plan and Issuer Data and Reporting (SPIDR) Salesforce instance and the Marketplace Eligibility & Enrollment Group (MEEG) Salesforce Community Portal instance in compliance with CCIIO's CCRMS Salesforce system security protocol boundary. Along with the MATS, SPIDR, and MEEG Community Salesforce instances, this work will also continue to support development, system integration, implementation and maintenance across multiple CMS IT systems to create stronger consumer No Surprises Act (NSA) protections related to health care pricing and billing in the newly created IDR and PPDR processes.

As with the Affordable Care Act (ACA), Congress also recognized the need for stronger consumer and stakeholder protections related to pricing and billing in health care through the passing of Title I (No Surprises Act) and Title II (Transparency) of Division BB—Private Health Insurance and Public Health Provisions-- of the Consolidated Appropriations Act, 2021, collectively referred to throughout as the No Surprises Act. The NSA was signed into law on December 27, 2020, and established new protections for consumers related to price transparency and surprise billing in health care. For example, plans and issuers are required to provide an Advance Explanation of Benefits prior to scheduled services to include, among other things, a good-faith estimate of the cost of the service. They must also offer price comparison information to allow enrollees to compare the cost sharing for items and services furnished by any participating provider. And it introduces new requirements on providers to ask about the individual's insurance coverage status and to provide a good-faith estimate of the expected charges when an individual schedules an item or service. As a result, CMS is developing a patient-provider dispute resolution (PPDR) process for uninsured individuals who are billed substantially more than estimated charges.

The NSA also takes significant steps to end surprise billing by building on existing provisions that require group health plans and issuers of group and individual health insurance coverage to cover emergency services without prior authorization, and at the same cost sharing for out-of-network services that applies to in-network services. The legislation also sets forth methodologies for determining the out-of-network rate that plans and issuers must pay providers, including air ambulance providers, for claims subject to surprise billing protections. If the plan or issuer and provider cannot agree on an amount during a 30-day negotiation period, the parties *may choose* to enter an independent dispute resolution (IDR) process.

The NSA directed CCIIO to implement a federal IDR process to protect against surprise medical bills. This task order will work with CMS and other stakeholders, including the Departments of the Treasury and Labor, to design, develop, and implement an automated federal independent dispute resolution (IDR) process; collect data on user fees from individuals who use the federal IDR process and pass that data to the CCIIO Risk Adjustment and Re-Insurance (RARI) Salesforce instance; certify IDR entities (IDREs) to carry out disputes resolutions; and collect quarterly data on group health plans or health insurance issuers offering group or individual health insurance coverage or types of such plans or coverage that have a pattern or practice of routine denial, low payment, or down-coding of claims, or otherwise abuse the 90-day period.

In conjunction with the IDR process, Congress passed a critical mandate for CCIIO to stand up a No Surprises Help Desk (NSHD) for consumers and providers to submit complaints through the CMS PPDR process. This task order will continue the development and maintenance of a complaints helpdesk platform to intake consumer, provider, and issuer complaints and information that includes an email-to-case inquiry resolution (IR) solution and Application Program Interfaces (API) to CMS stakeholders for inquiry review and resolution; and the implementation of the NSHD with an inContact call center solution that connects to the MATS Salesforce instance, which serves as the Department of Health and Human Services (HHS) complaints system of record.

Required Knowledge and Experience

CMS seeks a knowledgeable, experienced contractor to provide technical support that will aid CCIIO in providing ongoing technical assistance to the in-person assister community, Salesforce system IT development, system integration, and support across multiple CMS components and their IT systems, and NSA technical and operational support, including for the IDR process and the PPDR process. At a minimum, the following knowledge and experience are required:

- Extensive knowledge and experience in the ACA and Marketplaces, commercial individual and small group health insurance markets, Medicaid, CHIP, and other relevant insurance programs in States across the country.
- Extensive knowledge of the NSA, the IDR process, the PPDR process, NSA investigations, and complaints.
- Development of 508-compliant in-person technical support materials.
- Development of interactive, mobile-compatible WBT material.
- Development of mobile compatible, simulation-based microlearning modules.
- Development and maintenance of centralized electronic email inbox operations for many workflows.
- Development and maintenance of electronic program application processing for Marketplace and NSA programs.
- Development and maintenance of secure, 508-compliant, custom Salesforce community portals designed with Salesforce Lightning Web Components for 30,000+ users.
- Development and maintenance of a Salesforce Community to receive, intake, and process large volumes of data consisting of up to 250,000 records per intake using SOAP (Simple Object Access Protocol) and REST (Representational State Transfer) APIs application programming interface and Electronic File Transfers (EFT).
- Development and maintenance of Salesforce Communities that can receive millions of unstructured data files monthly and can handle surges 1000% higher than anticipated.
- Development and maintenance of a telephonic system integrated seamlessly with Salesforce to intake, triage, and resolve stakeholder complaints.

- System integration and user onboarding with CMS Identity Management through CMS IDM.
- Development and maintenance of many Salesforce APIs and coordination with other contractors and Agencies.
- Maintenance of systems in compliance with CMS Security requirements and maintenance of a federal Authority to Operate (ATO).
- Development and maintenance of data processes that involve running advanced custom algorithms that leverage geocoding to assign consumers losing health coverage to the nearest assister organization for direct outreach and enrollment assistance through the Marketplaces.
- Support of more than 150+ development releases on average annually.
- Development and maintenance of Salesforce workflows to support audits, investigations, and other compliance-related operations.

Skills required of the contractor will include:

- Expertise or experience with the ACA, the NSA, and regulatory and sub-regulatory guidance.
- Experience in developing, adapting or updating large, interactive, mobile compatible WBTs.
- Expertise or experience with in-person assister activities and needs.
- Expertise or experience developing fact sheets, frequently asked questions, Standard Operating Procedures, best practices, peer mentoring communities.
- Expertise providing coaching and consultation to assister organizations.
- Expertise or experience developing or updating multiple automated Salesforce applications simultaneously.
- Expertise or experience maintaining multiple Salesforce Orgs within a single Authority to Operate (ATO) security boundary.
- Expertise or experience maintaining dozens of APIs to other CMS systems.
- Expertise or experience with mass communication and coordination to multiple stakeholder groups.
- Expertise or experience with Marketplace and NSA inquiry response.
- Expertise or experience maintaining large data repositories using Amazon Web Services Cloud (AWS) S3.
- Expertise or experience supporting the security, scanning, and storage of millions of data files.
- Experience integrating Salesforce Orgs across multiple CMS IT systems,

Capability Statements

In response to this Notice, CMS is seeking respondents' capability statements. Capability statements are NOT quotes and do not address price/cost. Industry is merely asked to provide information, that demonstrates their expertise and capacity to meet the requirements of this work as a prime contractor, with subcontractors, as they deem necessary.

No marketing literature should be submitted in response to this SSN. **A mere statement that the contractor has a required capability is not sufficient.**

For the purposes of responding to this notice, respondents should address their capability.

When answering the following questions, respondents must provide specific and clear examples of same or similar work performed that demonstrates the required capabilities and the level of effort that was involved:

1. Describe your HIPAA compliance expertise and experience handling PII and PHI by citing specific examples of same or similar work that you recently performed. Identify your role, the type of data involved (PII/PHI), the HIPAA-related requirements applied (e.g., Privacy Rule, Security Rule, Breach Notification), and the key controls/deliverables implemented to ensure compliance.
2. Describe your Affordable Care Act (ACA) expertise and experience by citing specific examples of same or similar work you recently performed. Identify your role, and the ACA regulations and sub-regulatory guidance applied. At a minimum, include examples covering enrollment support; training development / delivery; stakeholder engagement and outreach; ACA policy support; health plan correction processing; and ACA-related data analysis and reporting.
3. Describe your knowledge of and prior experience with the NSA as well as related regulations and sub-regulatory guidance. This should include, at a minimum, stakeholder outreach and education, the IDR process, the PPDR Process, the IDR Entity application process and support, NSA data analysis and reporting, NSA compliance investigation support, and NSA policy expertise.
4. Describe your knowledge and experience building and maintaining multiple Salesforce environments, including the size(s) and type(s) of Salesforce Orgs you have supported; how you incorporated Tableau with Salesforce; how you maintained the underlying database; and how you met CMS Security requirements. Also describe your experience maintaining dozens of APIs integrating Salesforce with other CMS systems. Finally, summarize the depth and composition of your Salesforce-certified workforce, including representative certification categories and your approach to maintaining and expanding certified talent.
5. Describe your knowledge of and prior experience maintaining a secure FISMA Moderate system, including your experience with the federal Authority to Operate (ATO) process while managing more than 100 development releases annually under a single ATO. The response should include specific ATO(s) you have supported and your role in the ATO activities. Also describe how you managed millions of unstructured data artifacts, and your experience maintaining large data repositories in AWS S3 to store, secure, and govern that data.

6. Describe your knowledge with onboarding thousands of users simultaneously with user authentication via CMS' IDM system and Experian. FSSSO system users are required to pass Experian identity-proofing before accessing FSSSO systems. The response should include a recent example or examples of how you have onboarded thousands of users at a concentrated time through IDM and Experian.
7. Describe your knowledge of and prior experience working with in-person Assisters. The response should provide your role working with Assisters.
8. Describe your knowledge and prior experience creating or updating large web-based training (WBT) materials that are compatible with a third-party LMS and meet mobile requirements, and include a recent example of the training artifacts delivered (e.g., WBT modules plus supporting fact sheets, FAQs, SOPs, best-practice guides, and peer-mentoring/community resources)..
9. Describe your prior project management experience that includes a proven track record of keeping to a schedule, managing multiple tasks and multiple development teams, responding quickly to ad hoc requests that require immediate attention due to policy or regulatory changes, and implementing a scalable surge / onboarding staff to meet changing priorities within a short period of time.

Responses Requested

Please submit your responses **no later than 11:00am on March 20, 2026**, to Kelly.Housein@cms.hhs.gov and Megan.Famodu@cms.hhs.gov in the following format:

- Microsoft Word (or PDF) document with page size 8.5 by 11 inches, with a minimum of 1" margins.
- Times New Roman Font Size 11 with no less than single spacing between lines.
- **Submissions shall not exceed 10 pages.** Cover Page is included in the total page count.

Cover Page

Vendors shall include the following Business information on the Cover Page:

- a) SSN number, title, and date of issuance
- b) UEI
- c) Company Name
- d) Company Address
- e) Company Point of Contact, Phone and Email address
- f) Current GSA Schedules/Categories appropriate to this effort
- g) NAICS code under which the company conducts business as a small business and socio-economic status of the small business as validated via the System for Award

- Management (SAM). All potential offerors must be registered in SAM located <https://sam.gov/content/home>
- h) Number of full-time employees

Confidentiality

No proprietary, classified, confidential, or sensitive information should be included in your response. The Government reserves the right to use any non-proprietary technical information in any resultant solicitation(s).

Disclaimer and Important Notes

This notice does not obligate the Government to award a task order or otherwise pay for the information provided in response. Respondents are advised that the Government is under no obligation to acknowledge receipt of the information received or provide feedback to respondents with respect to any information submitted.

Teaming Arrangements

Firms seeking to respond to this notice as a team or to rely on subcontractors to perform any portion of the work must include the above-requested information and certifications for each entity on the proposed team or each proposed subcontractor. Responses must clearly indicate the nature of the teaming arrangement.

The contractor and any subcontractors must not have any relationships or arrangements through business operations or its employees that could be considered as possibly lessening the company's objectivity concerning any aspect of the proposed tasks. If such relationships or arrangements exist, contractors or subcontractors shall be required, during the procurement process (if one occurs), to identify potential conflicts of interest and discuss how the conflicts will be addressed and mitigated.

Similar Situated Entities

Description of how the respondent will provide at least 50% of the cost of labor (including indirect charges) required to perform the services. In the event of conflict between FAR clause 52.219-14 and 15 U.S.C. § 657s, for purposes of the limitations on subcontracting, CMS will follow 15 U.S.C. § 657s.